Medical Conversation Analysis Report

Generated on: 2026-09-12 18:24:15

Visit ID: visit-95a3e6c8

Primary guideline: NICE NG136 / JNC-8

Conversation History

patient: Hello, doctor. How are you? Hi, Ankit. What is going on?

Analysis and Insights

Key Points

- Chief complaint: Not specified
- Assessment: No clinical discussion occurred
- Top diagnosis: Essential Hypertension (confidence 0.736)

Action Items

- Continue Lisinopril 10mg, evaluate Amlodipine 5mg adjunct.
- Obtain: Home BP 24hr log

Patient Concerns

• None documented.

Medical Terms

• None documented.

Follow Up Questions

• None documented.

Symptoms

• None documented.

Medications

• None documented.

Diagnosis Considerations

- Essential Hypertension
- Tension-Type Headache
- White-Coat Hypertension
- Caffeine-Withdrawal Headache
- Secondary Hypertension (renal)

Risk Factors

• None documented.

Differential Diagnosis

• Essential Hypertension (74%)

Rationale: Persistent elevated BP, on Lisinopril, strong pattern match.

• Tension-Type Headache (68%)

Rationale: Afternoon onset, moderate severity, correlates with screen-time pattern.

• White-Coat Hypertension (63%)

Rationale: Home readings infrequent; clinic readings consistently higher.

• Caffeine-Withdrawal Headache (55%)

Rationale: Timing consistent. Caffeine history not yet confirmed.

• Secondary Hypertension (renal) (48%)

Rationale: No renal workup documented. Rule out if BP control worsens.